

Numbers and claims ledger

Use rule: A number may go on a slide only with its wording and qualifier below. “Synthetic simulation” numbers describe an experiment, not patients helped. “Synthetic demo” screen values describe the seed, not a district. If a slide has room for fewer words, remove the number rather than remove its qualifier. The dated project files are the record of what was built; the older [full PLAN](#) describes additional targets that must remain **NOT BUILT**.

External problem numbers allowed

Value	Exact slide wording / meaning	Original source: title · publisher · date	Where in our files	Slide
10.1 crore	“India: an estimated 10.1 crore people with diabetes in 2021.” A 2021 population projection, not current diagnosed or PHC patients.	“Metabolic non-communicable disease health report of India: ICMR-INDIAB-17” , <i>The Lancet Diabetes & Endocrinology</i> , 2023.	PLAN \$12 ; slide brief	2
31.5 crore	“India: an estimated 31.5 crore people with hypertension in 2021.” May overlap with diabetes; do not add the two counts .	ICMR-INDIAB-17 , <i>The Lancet Diabetes & Endocrinology</i> , 2023.	PLAN \$12	2
13.6 crore	Optional: “Estimated people with prediabetes in India, 2021”; do not present as Saathi’s treatment population.	ICMR-INDIAB-17 , <i>The Lancet Diabetes & Endocrinology</i> , 2023.	PLAN \$12	Optional 2
Up to 16 of 20 (80%)	“At some test-checked UP PHCs , up to 16 of 20 sampled drugs were never available during 2018–22.” A maximum in audited facilities, not a UP or national rate.	Report No. 8 of 2024: Public Health Infrastructure and Management of Health Services in Uttar Pradesh, chapter IV , Comptroller and Auditor General of India; tabled 19 Dec 2024.	PLAN \$12	2
Up to 1,459 days	Optional: “Some sampled drugs were out for up to 1,459 days at test-checked UP CHCs.” Do not generalise.	CAG UP report, chapter IV , CAG, 2024.	PLAN \$12	Optional 2
66; 71–82%	Optional, only with full qualifier: “At GMC Meerut, 71–82% of 66 sampled essential drugs were unavailable for more than four months in the audited years 2018–21.” Hospital sample, not PHC rate.	CAG UP report, chapter IV, Table 4.4 , CAG, 2024.	PLAN \$12	Optional 2
74.97 crore; over 8.64 crore; 31 states/UTs	Optional scale context: “As of Dec 2025, the National NCD Platform reported 74.97 crore beneficiaries enrolled and over 8.64 crore under treatment for hypertension or diabetes across 31 states/UTs.” These are platform figures, not Saathi reach.	“India’s Digital Public Infrastructure: Setting a global benchmark for population-scale DPI” , Press Information Bureau, 6 Mar 2026.	PLAN \$12	Optional 10
Over 10.40 lakh	Optional: “PIB reported over 10.40 lakh ASHAs nationally.” It says nothing about smartphone ownership or app access.	“The Last-Mile Architects of Rural Transformation” , Press Information Bureau, 20 Sep 2026.	PLAN \$12	Optional 10
36.3%; 16.7%	Optional: “Among surveyed adults with self-reported diabetes, 36.3% of the analysed group met glycaemic target; 16.7% reported self-monitoring blood glucose.” Neither measures medicine stockouts.	“Achievement of guideline recommended diabetes treatment targets and health habits in people with self-reported diabetes in India (ICMR-INDIAB-13)” , <i>The Lancet Diabetes & Endocrinology</i> , 2022.	PLAN \$12	Optional 2

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37.9%	Optional: “In an analysis of India’s 2017–18 national survey, both the inpatient and outpatient diabetes-care groups had 37.9% catastrophic health spending at the 10% consumption threshold.” This is not specific to medicine stockouts .	“Financial burden of seeking diabetes mellitus care in India: Evidence from a Nationally Representative Sample Survey” , <i>Health Care Science</i> , 4 Oct 2023.	PLAN §12	Optional 2 notes only
89.9%	Optional, only with sample boundary: “Among known-diabetes participants aged 50+ in selected districts, 89.9% had never had a retina examination.” Not a national all-age rate, and not a refill metric.	“Prevalence of diabetic retinopathy in India: Results from the National Survey, 2015–19” , <i>Indian Journal of Ophthalmology</i> , 2021.	PLAN §12	Roadmap notes only
4,915; 169	Optional research context: “A systematic review screened 4,915 QML studies and found 169 eligible; it found no consistent advantage over classical methods.” Not a Saathi experiment.	“A systematic review of quantum machine learning for digital health” , <i>npj Digital Medicine</i> , 2025.	PLAN §12	Roadmap notes only

CAG nuance: The audit also says the DVDMS sub-store module was used for indenting only at its test-checked facilities. That is a bounded audit finding; it does not mean DVDMS is absent nationally. Source: [CAG UP report, chapter IV](#); [PLAN §12](#).

Our measured and demo numbers allowed

Value	Exact slide wording / meaning	Original source: title · publisher · date	Where in our files	Slide
1 synthetic district; 3 blocks; 6 PHCs; 60 patients; 4 drugs; 90 days	“The seeded demo has one fictional district, three blocks, six PHCs, 60 synthetic patients, four drugs and a 90-day history.” Do not imply coverage or representativeness.	<i>Saathi seed and case contract</i> , project team, 28–29 Sep 2026.	HANDOFF, notes	Optional 4/10
5 steps; 4 roles	Diagram grouping for one case, not a count of all states: report; verify; review/approve; move/hand over; patient confirm. Roles: patient, ASHA, pharmacist, district officer.	<i>Saathi shared contract</i> , project team, 29 Sep 2026.	CONTRACT	1/4
30 tablets; Nayagaon donor	“In Ramesh’s synthetic seed , Nayagaon PHC is the eligible donor for a 30-tablet metformin request.” Other runs may differ.	<i>Saathi seed realism notes</i> , project team, 29 Sep 2026.	notes, S13	Optional 5
7 days; 14 days	“The demo warns at under 7 days critical and under 14 days low.” These are product rules, not clinically validated thresholds.	<i>Saathi shared contract</i> , project team, 29 Sep 2026.	CONTRACT	Optional 8
14-day reserve; >30 days after arrival	“A transfer draft protects the donor’s 14-day safety stock and uses batches valid more than 30 days after arrival.” Product rules, not government policy.	<i>Saathi shared contract</i> , project team, 29 Sep 2026.	CONTRACT	Optional 6/9
227.58 → 3.33 unmet patient-days	“In the baseline synthetic simulation , dispensing-only averaged 227.58 unmet patient-days per run; prescription-only averaged 3.33.” Never call this the shipped default’s reduction . Rounded 228→3 is acceptable only with those method labels.	Forecast evaluation , Saathi project team, 29 Sep 2026 report.	README, HANDOFF	7

Value	Exact slide wording / meaning	Original source: title · publisher · date	Where in our files	Slide
0.333 false-alert rate = 1 in 3	“Prescription-only had false alerts in 1 in 3 baseline simulation runs.” Denominator: runs, not alerts, patients, or facilities. Dispensing-only and experimental combined: 0.000 in that baseline.	<i>Forecast evaluation</i> , Saathi project team, 29 Sep 2026 report.	README , forecast report	7
389.19 vs 0.25 overstock units	“In the baseline simulation, prescription-only averaged 389.19 overstock units per run vs 0.25 for dispensing-only.” This is simulated inventory.	<i>Forecast evaluation</i> , Saathi project team, 29 Sep 2026 report.	README , forecast report	7
3.81 vs 4.98 units/day MAE; both 227.58 unmet patient-days	“Experimental combined forecast had lower baseline mean absolute error than dispensing-only, but both had the same simulated unmet patient-days.” The combined experiment is not the shipped <code>max</code> default.	<i>Forecast evaluation</i> , Saathi project team, 29 Sep 2026 report.	README , CONTRACT v0.4	7
17.51 vs 3.57 units/day MAE; 236.45 vs 0.97 overstock units	Optional counterexample: “With stale prescriptions, experimental combined was worse than dispensing-only on error and overstock.” Same synthetic simulation.	<i>Forecast evaluation</i> , Saathi project team, 29 Sep 2026 report.	README	7 notes
12/12; 5.96 seconds	“In a paced API test, all 12 synthetic Hindi TTS clips got live Gemini responses; medicine, requested quantity, home-supply days and spoken date matched scripts in 12/12; mean API latency was 5.96 seconds.” Not real-speech accuracy or browser latency.	<i>Live voice evaluation</i> , Saathi project team, 29 Sep 2026.	README , audio report	Optional 6 or notes
1 real clip; 10.33 seconds	“One real, non-refill WhatsApp Ogg/Opus clip was classified correctly on a standalone live retry in 10.33 seconds; an earlier paced call fell back.” No real refill clip was scored.	<i>Live voice evaluation</i> , Saathi project team, 29 Sep 2026.	README	Q&A only
26/30 live answers; 4 failures	“A text-only test of 30 synthetic Hindi/Hinglish transcripts got 26 live answers and four provider failures; it did not test speech recognition.” Across all 30: medicine 0.867, requested quantity 0.833, home days 0.867, negation 0.967.	<i>B16–B19 continuation</i> , Saathi project team, 29 Sep 2026.	README , notes	Q&A only
~0.4 s; ~9 s; ~5.5 s	“At the 29 Sep live handoff, overview took about 0.4 s, transfer draft about 9 s, and reset about 5.5 s.” Snapshot observations, not a reliability or speed guarantee.	<i>Saathi HANDOFF</i> , project team, 29 Sep 2026 ~10 AM.	HANDOFF	8 notes
11.8 ms local first overview; 3.1 ms local warm; 223.1 ms local reset	Optional engineering detail: measured in-process on one Mac with a temporary SQLite file; not Render latency .	<i>District overview cold path and reset speed</i> , project team, 29 Sep 2026.	notes	Q&A only
One block, about 5–6 PHCs	“Proposed supervised pilot unit: one block with about five to six PHCs.” A planning assumption, not an agreed site.	<i>Plan v3</i> , project team, 28–29 Sep 2026.	PLAN.\$0.6..\$Z	10

Forecast method footnote for slide 7: In the simulation, mean absolute error compares units/day with independently generated total patient need. Unmet patient-days and false alerts use PHC requests. Orders arrive after seven days and are sized to 130% of each forecast. These are experiment settings, not Saathi operating rules. Source: [forecast report](#).

Claims with no numeric headline

Say	Why / source
"Known stockout days are excluded from the dispensing series; current demo planning uses <code>max(cohort_need, dispensing_forecast)</code> without summing."	README , CONTRACT .
"Gemini extracts voice fields and may word a checked transfer rationale; OR-Tools chooses a feasible donor; people confirm and approve."	README , CONTRACT .
"DVDMS exists; Saathi's proposed value is patient-level refill signals and closed-case follow-through alongside existing stock systems."	CAG UP audit , PIB DVDMS description , PLAN §2 . Integration is NOT BUILT .
"Real patient speech and field impact still need testing."	audio report , README .

Never say → say instead

Never say	Say instead
"First", "novel", "only system"	"A patient-to-district refill loop we built as a synthetic prototype." No comparative novelty audit was done.
"Real patients use Saathi" or "tested with ASHAs"	"All app patients and stock are synthetic; field testing is proposed." The one real voice clip was a separate non-refill API test.
"Clinically validated", "medical diagnosis", "predicts blood sugar from a photo"	"No clinical claim; meal/glucose work and clinician review are NOT BUILT ."
"Works for all Indian languages", "Hindi speech accuracy proved"	"Hindi/Hinglish scripted TTS did well in a small API test; real refill speech and dialects are barely tested."
"228 patients helped", "stockouts cut 99%", "Saathi cut unmet days from 228 to 3"	"Prescription-only cut simulated unmet patient-days from 227.58 to 3.33 versus dispensing-only, with false alerts and overstock; shipped <code>max</code> has no proven field effect."
"Our combined AI method cut missed refills"	"Experimental combined lowered baseline error, but simulated unmet patient-days stayed at 227.58."
"AI moves stock automatically"	"Rules check a draft; an officer approves; staff dispatch and receive."
"National real-time stock coverage"	"One synthetic district demo; authoritative stock import and state integration are roadmap."
"ABDM compliant", "privacy solved", "secure login"	"Demo role picker is not secure identity; consent, access control and integration work are pilot gates."
"Instant AI" or "works fully offline"	"Voice and transfer drafting need connectivity; supported writes can queue for later sync."
"₹X per patient/district", "X% ASHAs have phones", "X% PHCs use paper"	"[UNVERIFIED — do not use] until measured or sourced for the intended geography."

[UNVERIFIED — do not use]

- **₹ cost per district, per patient, or national rollout:** no priced staffing, hosting, integration, device, support and training model exists in the supplied files.
- **National ASHA smartphone access rate** and **national share of PHCs using paper registers:** [PLAN §13](#) explicitly leaves these unconfirmed.
- **37.9% catastrophic spending as a stockout consequence:** the PLAN cites a diabetes-care spending analysis, but it does not establish stockouts as the cause; do not use for this deck's supply claim.

- **23 crore Scan & Share tokens; Indian thali photo accuracy; any field reduction in stockouts, missed doses or private spending:** unverified or unmeasured in [PLAN §13](#).
- **HealthGrid AI feature-by-feature superiority or absence:** no verified comparison in the project files. Treat overlap as real and answer on the workflow Saathi actually demonstrates.